Field Guide 01

The Clinician's AI Safety Check

A brief pause before using AI at work.

Before | During | After

Version 2.0, November 2026. Your organisation's policy always comes first.

Before you open the tool

The quick check

Three questions. Any answer that needs action tells you what to do.

Question 1 of 3

1. Tool and task

Is this specific AI tool, account and intended use permitted by my organisation?

Permission is specific. A tool approved for one task, or through one account, isn't approved for everything.

2. Information

Am I only using information permitted for this tool and workflow?

Include attachments, screenshots and anything you might paste in later.

3. Identifiability

Could these details, alone or together, identify a person?

Removing a name isn't enough. Age, condition, place and circumstances can add up. **See how**.

Why removing a name isn't enough

The details add up

Each detail on its own fits a lot of people. Watch what happens when they overlap.



Three fictional details

1. In their eighties
2. Admitted after a fall this week
3. Lives above the only shop on their street

These details are invented for this lesson. There's no safe number of details: what identifies someone depends on context and on what else is known.

Read once, keep for reference

The full guide

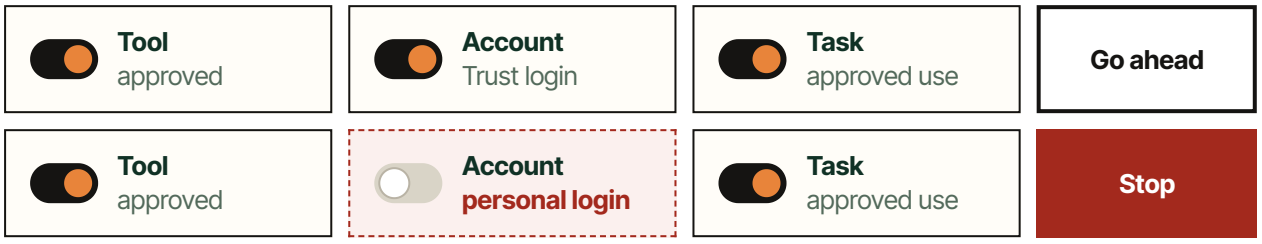
Nine checks across three stages. You don’t need to read all of this every time. The quick check is for that.

Before

Before you open the tool

Check 1. Where: approval

Is this exact tool, account and environment approved for my intended task?



All three have to be on. One off is enough to stop.

Approval is specific. The same tool can be approved through a Trust account and not a personal one, or for drafting and not for clinical decisions. If you can’t point to the permission, assume you don’t have it.

Who can tell you

Check 2. What: information

What am I entering, and is every data type and detail permitted and necessary?

Go through what you're about to enter, one type at a time: text, files, images, numbers. Leave out anything the tool doesn't need to do the task. Often the task doesn't need the person at all.

What you know		What the task needs
83 years old	→	In their eighties
Stroke three weeks ago	→	Recovering from a stroke
Lives alone on Elm Street	→	Lives alone
Seen yesterday afternoon	-- ×	Not needed
Referred by Dr A. Khan	-- ×	Not needed
Ward 4B, Northfield General	-- ×	Not needed
Walks with a frame, one person helping	→	Uses a walking frame

→ made general -- × left out

Three of seven details were left out completely, not disguised. The right-hand side is less identifiable, not automatically anonymous: still enter only what your organisation permits for this tool.

Check 3. Why: purpose

Is the purpose appropriate and authorised, and could details identify someone alone or together?

Drafting a general leaflet	Summarising a guideline for yourself	Drafting a letter about one patient	Recommending care for one patient

General support **Needs specific approval**

Lower risk does not always mean approved for use.

Think about what you're asking AI to do. Drafting a general leaflet is different from helping make a decision about one patient. Then check whether the details could identify someone when put together.

The details add up

During

While you're using it

Check 4. Minimise

Am I staying within permitted information boundaries, including attachments, screenshots and follow-up prompts?

AI tools ask follow-up questions, and each one invites a little more detail. This is prompt creep. Try it:

Practice chat. Not a real tool — **Fictional**

You

Can you draft a simple home exercise programme for someone recovering after a fall?

AI tool

Happy to help. To tailor it, can you tell me the patient's full name, exact age and which ward they're on?

Check 5. Boundaries

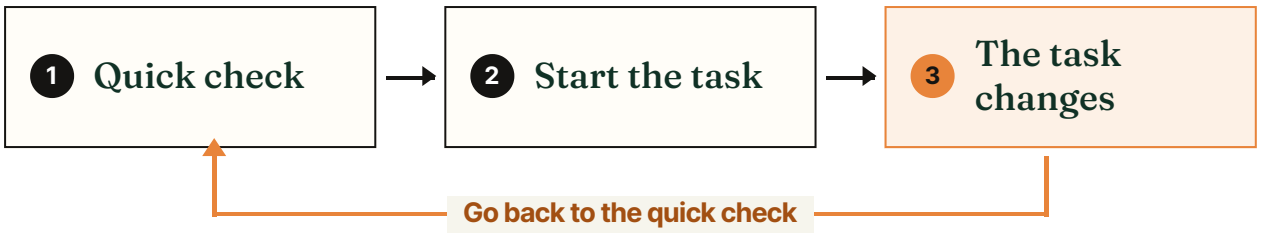
Am I keeping AI within its authorised supporting role and retaining human clinical judgement?

AI can	Your judgement	Only you
Draft Summarise Suggest wording		Decide Sign off Answer for it

Drafting support is not clinical decision support. If the output starts recommending what to do for a specific patient, you've moved into a different kind of use, which may need different approval.

Check 6. Reassess

If the task changes, AI asks for more data or a concern arises, should I stop and seek advice?



The check you did at the start covered the task you started with. If the task grows, run the quick check again.

> **AI confidence is not evidence.**

After

Before you save, share or use the output

Check 7. What changed?

Did AI invent, omit, distort or overstate anything clinically important compared with the source?

Your source	AI version
Walk with a frame and one person supervising[1]. Avoid stairs until the next review.[2] Daily practice may help[3] with confidence. Review in two weeks.	Walk independently[1] with a frame. missing[2] Daily practice will restore[3] confidence. Review in two weeks. Most patients recover fully within a month.[4]

1. **Distorted** Supervision became "independently".
2. **Left out** The stairs warning disappeared.
3. **Overstated** "May help" became "will restore".
4. **Invented** A recovery claim with no source.

It reads well. It's still wrong in four places, and only one of them is an invented fact.

Errors aren't only invented facts. Look for what's missing, what's been softened or strengthened, and what now means something slightly different.

Check 8. Verify

Have I checked relevant facts, figures, medications, links, evidence and references against suitable sources?

Links	References	Doses	Figures
Open every one	Find every one	Check every one	Trace every one

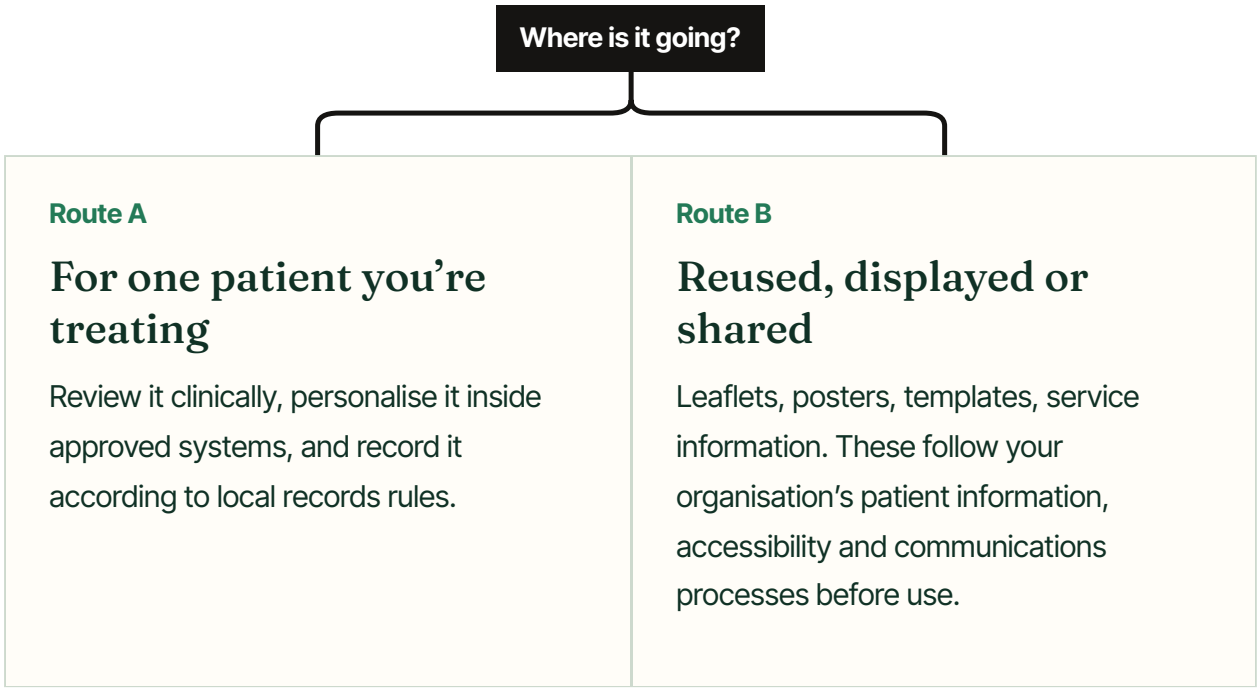
Open every link. Find every reference. Check every dose and number against a source you'd trust without AI.

Can you spot an invented reference?

Check 9. Approve and handle

Has the output followed the appropriate professional review, patient-specific or wider-distribution route, and storage and versioning rules?

Where the output goes decides the route. It's one route or the other, not both:



Either way: date it, give it a version, and store it where your organisation says it belongs.

One procedure, used everywhere

If something goes wrong

If you've shared something you shouldn't have, or relied on an output that may be unsafe:

1 **Stop**

Stop further disclosure, sharing or reliance on potentially unsafe output.

2 **Report**

Promptly use your organisation's approved incident channel. Contact the relevant manager, IG or data protection, security or clinical safety team as required.

3 **Record**

Document the facts, time, tool and account, possible exposure and any patient impact, only in approved systems.

4 **Contain**

Follow instructions about access, retention, deletion and evidence preservation. Don't assume deleting the chat reverses the disclosure.

5 **Cooperate**

Follow the response and any patient-safety actions. Your organisation decides on any regulatory notifications.

An AI-generated clinical error may need clinical safety escalation even when no data breach has happened.

Before you need them

Who to ask

Most people find out who these people are when something has already gone wrong. Find out now.

Your line manager

Where to start. They can tell you local practice and who the right specialist is.

IG or data protection

What information can go into which tool, and what to do after a disclosure.

IT or security

Which tools and accounts are approved, and how they're set up.

AI governance

If your organisation has one: approved uses and how new uses are assessed.

Clinical safety officer

Clinical risk from digital tools, including errors in AI output.

Questions worth asking

- Which AI tools and accounts are approved here, and for which tasks?
- What information am I allowed to enter into each one?
- Who approves a new use of an approved tool?
- How do I report an AI-related mistake, including a clinical error with no data breach?
- Where should AI-assisted outputs and templates be stored?

Governance

About this guide

Status	Current version.
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Scope

This is general professional guidance for healthcare staff. It isn't a substitute for your organisation's policies, local clinical governance or advice from your information governance lead, and it doesn't approve any tool or use. Where local policy differs, follow local policy.

Clinically Intelligent is written in a personal capacity. It isn't affiliated with, endorsed by or representative of any NHS organisation.

Version history

Version 2.0, current

November 2026

Rebuilt as a quick check and a full guide. Check 3 restored, one incident procedure, downloads and an accessible web version.

You're reading this version.

Version 1.0, archived

May 2026

The original nine-page PDF, kept as a record of what was published.

View archived version 1.0

Older versions are kept as records only. Always use the current version for practical guidance.

Sources

- NHS England: AI guidance for health and care professionals
- NHS England: clinical risk management standards
- NHS England: digital clinical safety assurance
- ICO: can we identify an individual indirectly?
- ICO: how do we ensure anonymisation is effective?
- ICO: personal data breaches, a guide